



Philippine Integrated Disease Surveillance and Response

Case Investigation Form
Rabies (ICD 10 Code:A82)



Name of DRU:						Type: ☐ RHU ☐ CHO ☐ Gov't Hospital ☐ Private Hospital ☐ Clinic ☐ Gov't Laboratory ☐ Private Laboratory ☐ Airport/ Seaport							
Address:													
I. PATIENT INFORMATION:		Patient Number:	Patient's Last Name		First Name	Middle Name							
Current Address: (Specify House/Lot #, Street/Purok/Subdivision, Barangay, Municipality/City, Province, Region)						Sex: ☐ Male ☐ Female	Date of Birth:	MM	DD	YY	Age:	☐ Days ☐ Months ☐ Years	
Permanent Address: (Specify House/Lot #, Street/Purok/Subdivision, Barangay, Municipality/City, Province, Region)								MM	DD	YY			
District: HCPN								MM	DD	YY			
Patient Admitted? ☐ Yes ☐ No ☐ Unknown				Date Admitted/ Seen/ Consult	MM	DD	YY	Date Onset of Illness		MM	DD	YY	
Date of Report:		MM	DD	YY	Name of reporter:			Contact Nos.:					
Date of Investigation:		MM	DD	YY	Name of investigator/s:			Contact Nos.:					
II. EXPOSURE HISTORY:													
Type of exposure: ☐ bite ☐ scratch Affected site _____ ☐ saliva ☐ consumed meat ☐ Unknown ☐ Other, specify _____ Date of exposure: _____ Place of exposure: _____ Category of Exposure: ☐ Feeding/touching an animal ☐ Licking of intact skin(with reliable history and thorough physical examination) ☐ Exposure to patient with signs and symptoms of rabies by sharing of eating or drinking utensils ☐ Casual contact (talking to, visiting and feeding suspected rabies cases) and routine delivery of health care to patient with signs and symptoms of rabies ☐ Nibbling of uncovered skin with or without bruising/hematoma ☐ Minor scratches/abrasions without bleeding ☐ Minor scratches/abrasions which are induced to bleed ☐ All Category II exposures on the head and neck area are considered Category III and should be managed as such ☐ Transdermal bites(puncture wounds,lacerations,avulsions) or scratches/abrasions with spontaneous bleeding ☐ Licks on broken skin or mucous membrane ☐ Exposure to a rabies patient through bites,contamination of mucous membranes(eyes,oral/nasal mucosa,genital/anal mucous membranes) or Open skin lesions with body fluids through splattering and mouth-to-mouth resuscitation. ☐ Unprotected handling of infected carcass or ingestion of raw infected meat ☐ All Category II exposures on head and neck area Type of animal: ☐ dog ☐ cat ☐ bat ☐ Other, specify _____ Lab. diagnosis done? ☐ Yes ☐ No ☐ Unknown If Yes, result: _____ Animal status: ☐ domestic ☐ stray ☐ wild ☐ Other, specify _____													
III. VACCINATION HISTORY:													
Animal vaccination history: ☐ Vaccinated ☐ Unvaccinated ☐ Unknown				Patient History: Wound cleaned?: ☐ Yes ☐ No ☐ Unknown Patient given RIG?: ☐ Yes ☐ No ☐ Unknown (RIG is Rabies Immunoglobulin) Patient given rabies vaccine?: ☐ Yes ☐ No ☐ Unknown				Date vaccine started _____ Brand Name of Vaccine: _____ Route of Administration: ☐ IM ☐ Intradermal Post exposure completed ☐ Yes ☐ No					

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Deliberately providing false or misleading, personal information on the part of the patient, or the next of kin in case of patient's incapacity, may constitute non-cooperation punishable under the Republic Act No. 4132.



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IV. CLASSIFICATION AND OUTCOME:

FINAL CLASSIFICATION	
☐ Suspect Case	Alive
☐ Probable Case	Died
☐ Confirmed Case	Date died: ____/____/____

Case Definition/Classification:

Suspect Case

A person presenting with an acute neurological syndrome (encephalitis) dominated by forms of hyperactivity (furious rabies) or paralytic syndrome (dumb rabies) that progresses towards coma and death, usually by respiratory failure, within 7-10 days after the first symptom if no intensive care is instituted or as diagnosed by attending physician.

Probable Case

Suspect case plus history of contact with suspected or laboratory-confirmed rabid animal.

Confirmed Case

A suspected case that is laboratory confirmed.

Laboratory Confirmation:

-Detection of rabies viral antigens by direct fluorescent antibody test (FAT) or by ELISA in clinical specimens, preferably brain tissue (collected post mortem).

-Detection by FAT on skin biopsy (ante mortem).

-FAT positive after inoculation of brain tissue, saliva or CSF in cell culture, or after intracerebral inoculation in mice or in suckling mice.

-Detectable rabies-neutralizing antibody titer in the serum or the CSF of an unvaccinated person.

-Detection of viral nucleic acids by PCR on tissue collected post mortem or intra vitam in a clinical specimen (brain tissue or skin, cornea, urine or saliva).

-Isolation of rabies virus from clinical specimens and confirmation of rabies viral antigens by direct fluorescent antibody testing.

[Handwritten signatures]